

Women's Specialists of Clear Lake

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Shoulder Dystocia

A rare birth emergency where the baby's shoulder gets stuck -- our team is trained to respond quickly and safely

WHAT IS SHOULDER DYSTOCIA?

- A condition during **vaginal delivery** when the baby's shoulder gets stuck behind the mother's pelvic bone after the head delivers
- It is an **obstetric emergency** that requires quick, coordinated action by your delivery team
- It is **uncommon** and most often resolved successfully with specific maneuvers
- There are usually **no warning symptoms** for the mother -- it is recognized when the baby's shoulders do not deliver after the head

HOW WE TREAT IT

- ✓ We will ask you to **stop pushing** for a moment so we can reposition
- ✓ **McRoberts maneuver**: bringing your thighs up onto your belly to open the pelvis
- ✓ **Suprapubic pressure**: gentle pressure just above the pubic bone to free the shoulder
- ✓ Reaching into the vagina to **rotate the baby** or deliver the back arm
- ✓ Other position changes (such as hands-and-knees) if needed
- ✓ Most cases are resolved within **a few minutes** using these steps

REDUCING YOUR RISK

- ✓ **Manage blood sugar** carefully if you have diabetes or gestational diabetes
- ✓ Aim for **healthy pregnancy weight gain** as recommended at your visits
- ✓ Stay active with **provider-approved exercise** during pregnancy
- ✓ Attend all **prenatal appointments** so we can monitor baby's growth
- ✓ Discuss any concerns about **baby's size** with your provider

WHO IS AT HIGHER RISK?

- Babies estimated to be **larger than average** (macrosomia)
- **Diabetes** during pregnancy (gestational or pre-existing)
- **Excess maternal body weight** or excessive pregnancy weight gain
- A **previous delivery** complicated by shoulder dystocia
- Prolonged second stage of labor or operative vaginal delivery
- **Important**: Most cases happen with no risk factors at all -- it cannot be reliably predicted or always prevented

POSSIBLE COMPLICATIONS

- **For baby**: a fractured collarbone or arm bone -- usually heals well on its own
- **Brachial plexus injury (Erb's palsy)**: stretching of the nerves in the neck/shoulder causing arm weakness; **most cases recover** within weeks to months
- Brief drop in baby's oxygen during the stuck period
- **For mother**: vaginal or perineal tears, and heavier-than-usual bleeding
- Our team will examine you and your baby carefully after delivery

PLANNING A FUTURE PREGNANCY

- If you had shoulder dystocia before, the chance of recurrence is **low but increased**
- We will review the **details of the prior delivery** -- baby's size, severity, any injury
- A **planned cesarean (C-section)** may be recommended in some situations
- **Optimizing weight and blood sugar** before pregnancy lowers your risk
- We will create a **personalized delivery plan** with you

⚠️AFTER DELIVERY -- CALL US IF YOU NOTICE

- ⚠️ Heavy vaginal bleeding (soaking a pad in under an hour)
- ⚠️ Severe pelvic, vaginal, or abdominal pain
- ⚠️ Fever above 100.4 degrees F
- ⚠️ Foul-smelling vaginal discharge
- ⚠️ Baby is not moving an arm normally
- ⚠️ Baby's arm appears limp or in an unusual position
- ⚠️ Baby is not feeding well or seems unusually fussy
- ⚠️ Any concern about you or your baby's recovery

You Are in Experienced Hands

Shoulder dystocia is uncommon, and our team trains regularly to respond safely and quickly when it does occur. Most babies and mothers recover fully with no lasting effects. If you have a history of shoulder dystocia, diabetes, or concerns about your baby's size, please talk with us -- together we will make a delivery plan that protects you and your baby.